



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : رضا نبيل سالم عياد جمعان
المحافظة : القليوبية الحي / المدينة : طوخ
رقم البطاقة : 27712101403221 رقم التليفون : 01288339428
العنوان : اجهور الكبرى
العمر (تاريخ الميلاد) : 10-12-1977 المهنة : بدون عمل
تاريخ تسجيل الإستمارة : 23-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : -----
الاسم : د/ندى حسن الوظيفة : -----
الاسم : د/نرمين رمضان الوظيفة : -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----
قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: رضا نبيل سالم عياد جمعان National ID: 27712101403221	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الى Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



MDT FORM

Name:	Rida Nabil Salim	MRN:	1554
Age:	48 Y	ID:	27712101403221
Menopausal Sta	prem	Phone:	01221319844
NRS SCORE:	ps 1	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: ER: PR: HER2: KI67:

Sonomammography:

Mets Workup:

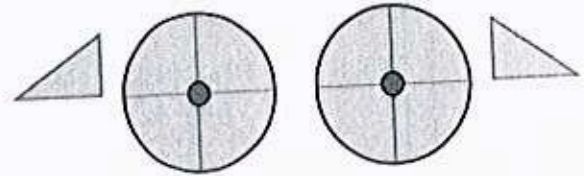
History:

premenopausal metastatic breast cancer to bone idc 3 er7/8 pr -ve her2 3+
ct4 n1 m1 on taxol ,herceptin pt rec 19 weeks taxol 6 herceptin iv cts 2/2026
free bs 3/2026 sd pt had grade 2 to 3 pn for supp tt and mdt

Recommendations:

MDT Outcome & Plan

ZOLADEX +AI
CONTINUE HERCEPTAN



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. نوري حسن	د. نوري حسن				

MDT FORM

١٩ / ١١ / ٢٠٢٨ G

Name	رانيا بيل سالم عيار	MRN	2607/28
Age	47	ID	٢٦٠٧ / ٢٨
Menopausal status	premenopause	Phone	0100 6075520
NRS SCORE		Marital status	married with
BMI		Family history	no 5 offspring
CO morbidity	no comorbidities.		

Clinical Data			
BX:	IDC G III	(LT) BC	R
ER:	7/8	PR:	0/8
HER2:	+3	Ki67:	45%
Sonomammography: left breast suspicious mass 4.7x2.2		Ps 0-1	
left axillary L.V. non palpable		T1c1, N1, M1	
Mets Workup:		TNM (De novo Metastasis to bone)	
CTs 25/9/2025 ⇒ No hepatic, Pulmonary			
But LS sclerotic lesion for bone scan.			
Bone Scan 6/11/2025 ⇒ active osseous lesions at sternum,			
Recommendations:		left scapular, lt sacral, and Both iliac bones.	
MRI brain ⇒ As patient is complaining of severe headache and blurring of vision.			

MDT OUTCOME & PLAN	
Taxol / Hercept I.V. 4-6 hours per week	Monitor heart / liver
(2 doses / week)	Consider beta-blocker for pain & psychology
	2 courses / month

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

atient Name : الأستاذ / رضا نبيل سالم عياد جمعان	Request No. : 13006998
atient No. : 27712101403221	Request Date : 06/09/2025
ath No. : 2747/25	Verification Date : 10/09/2025
ex : Female	Financial Category : جوستاف روسي - مبادره
ge : 47 Y	Department : Out Patient
eferreing Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Left breast 3 o'clock mass.
Nature of Specimen: Tru-cut biopsy, collected on 03/09/2025.

Gross Examination:

- Four grey white tissue cores, the longest was 1.5 cm long. Totally submitted.
- Technique:
Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:

- Histological type: Invasive duct carcinoma.
- Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- 2% Low grade ductal carcinoma insitu (Solid pattern).
- Angioinvasion: Absent.
- Perineural invasion: Absent.
- Microcalcification: Present.
- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 50% of tumor cells (score 7/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 45%.

Diagnosis:

- Left breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 3.
 - 2% Low grade ductal carcinoma insitu (Solid pattern).

Verified by :

Dr.Duaa Mohamed

Patient Name : الأستاذ / رضا نبيل سالم عياد جمعان	Request No. : 13006998
Patient No. : 27712101403221	Request Date : 06/09/2025
Path No. : 2747/25	Verification Date : 10/09/2025
Sex : Female	Financial Category : جوستاف روسي - مبادره
Age : 47 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

IPT:

- ER: Positive (score 7/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67%: 45%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed



Name: رطل نبيل سالم

Date: 29/6/2024 5

SONOMAMMOGRAPHY EXAMINATION OF BOTH BREASTS

female patient presented by right breast lump one year ago .

- * Scattered fibroglandular breast parenchyma (ACR b)
- * Left reteroalar UOQ, irregular, speculated iso dense mass with suspicious microcalcification seen with in, with diffuse increased skin thickness more at the reteroaeolar region.
- * No right speculated mass or suspicious micro calcifications.

By complementary ultrasound:

Left breast:

*4/5 o'clock irregular speculated, hypoechoic mass measuring 47x27mm it seen about 10mm from the nipple & 9 mm from skin the lesion seen highly shadowing with multiple tiny echogenic foci (microcalcification) showing long medial specule measuring 23mm connecting to another smaller similar lesion measuring 8.8x7.7 mm seen at the 6 oclock.

*Reteroaeolar diffuse increase skin thickness with echogenic fat interstitial edema lines no sizable collection.

*left axillary indeterminate lymph node seen diffuse cortical thicknening measuring 6.2 mm , effaced hilum still seen oval shaped measuring 15x6.8 mm .

*No pathologically enlarged infra/supra calvicular regions.

Right breast:

*No cystic or solid masses.

*Normal skin thickness & subcutaneous tissue.

*Normal nipple & areola complex.

* non specific axillary LNs

Opinion:

→ axilly LN by examet. ?

- left breast suspicious mass as described above (BIRADs 4C) for histopathological correlation .

- Normal right breast (BIRADS 1).

Dr sabah elnezamy MSC

Name : Reda Nabil Salem
Branch : Portable
Ref.Physician Prof: CAP

Date : 16-02-2026 02:53
Medical No. : GRI-121724
Ins.Company :

**MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL
RECONSTRUCTION: -**

Findings:

- Left breast ill defined retroareolar soft tissue mass measures 2.3x3.7 cm with skin thickening measures 9 mm.
- Left axillary lymphadenopathy measures 13x5.6 mm.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Upper abdominal cuts are unremarkable.

OPINION:

- Left breast ill defined retroareolar soft tissue mass for US correlation.

Name : Reda Nabil Salem
Branch : Portable
Ref.Physician Prof : CAP

Date : 16-02-2026 02:53
Medical No. : GRI-121724
Ins.Company :

Dr. Mai Seddik

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS: -

Findings:

- Bulky uterus for gynecological assessment.
- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

- Bulky uterus for gynecological assessment.

Dr Mai Mohamed MD

Patient Name: Reda Nabil Salem Ayad Gam'aan	Date : 25/09/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 47 Year(s)
Medical ID: 2607/2025	Pat. ID: 121724

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

***A known case of left breast cancer**

Chest: -

- Left breast architectural distortion with overlying skin thickening , with amximum thickness 11 mm , dilated tortous vessels are noted just below the skin .
- Multiple enlarged left axillary L.Ns , the largest 17 x11 mm.
- Few enlarged pulmonary nodules average 3mm.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No sizable hilar or mediastinal L. Ns .
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Abdomen & pelvis:

- The liver is mildly enlarged in size with homogenous CT texture. No focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

The scanned skeleton:

- L5 sclerotic lesion .
- No vertebral structural collapse or extra-osseous soft tissue component.

inion:

No hepatic , pulmonary metastatic deposits
L5 sclerotic lesion for correlation with bone scan.

Dr. Reham Khedr, MD

Name : Reda Nabil Salem Ayad

Date : 06-11-2025 01:28

Branch : Dokki

Medical No. : 243221

Ref.Physician Prof : Gostaf Roseh Hospital

Ins.Company : تعاقد مستشفيات

Tc^{99m} MDPSKELETAL SCINTIGRAPHY

History

- A 47-year-old lady, with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- Active osseous lesions seen at sternum, left scapula, left sacro-iliac and both iliac bones.
- No other abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- Few metastatic osseous lesions as described.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine